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Rigid vs. flexible dieting: association with eating disorder symptoms in nonobese women

Tiffany M Stewart¹, Donald A Williamson, Marney A White

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Abstract

The correlates of rigid and flexible dieting were examined in a sample of 188 nonobese women recruited from the community and from a university. The primary aim of the study was to test the hypothesis that women who utilize rigid versus flexible dieting strategies to prevent weight gain report more eating disorder symptoms and higher body mass index (BMI) in comparison to women who utilize flexible dieting strategies. The study sample included women who were underweight (29%), normal weight (52%), and overweight (19%). None of the women were obese, as defined by BMI>30. Participants were administered a questionnaire that measures Rigid Control and Flexible Control of eating. Body weight and height were measured and measures of eating disorder symptoms and mood disturbances were administered. Our results indicated that BMI was significantly correlated with rigid dieting and flexible dieting. BMI was controlled statistically in other analyses. The study found that individuals who engage in rigid dieting strategies reported symptoms of an eating disorder, mood disturbances, and excessive concern with body size/shape. In contrast, flexible dieting strategies were not highly associated with BMI, eating disorder symptoms, mood disturbances, or concerns with body size. Since this was a cross sectional study, causality of eating disorder symptoms could not be addressed. These findings replicate and extend the findings of earlier studies. These findings suggest that rigid dieting strategies, but not flexible dieting strategies, are associated with eating disorder symptoms and higher BMI in nonobese women.

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